

MOOD DISORDERS - OME Lesson

MAJOR DEPRESSIVE DISORDER (MDD)

- Criteria: 5 of 9 SIGECAPS symptoms for $\geq$ 2 weeks
- At least one of: depressed mood or anhedonia
- SIGECAPS: Sleep, Interest, Guilt, Energy, Concentration, Appetite, Psychomotor changes, Suicidality
- Prevalence: lifetime risk 15-20%, women 2x more than men
- Pathophysiology: serotonin, norepinephrine, dopamine dysregulation

MDD - Treatment

FIRST-LINE TREATMENT

- SSRIs: fluoxetine, sertraline, escitalopram (preferred first-line)
- SNRIs: venlafaxine, duloxetine (good for comorbid pain)
- Bupropion: no sexual side effects, avoid in seizure/eating disorder
- TCAs: amitriptyline (second-line, anticholinergic side effects)
- MAOIs: phenelzine (last resort; tyramine diet restriction required)
- Psychotherapy: CBT equally effective for mild-moderate MDD

BIPOLAR DISORDER

BIPOLAR I vs BIPOLAR II

- Bipolar I: full mania (≥ 7 days) + depression
- Bipolar II: hypomania (4-6 days) + depression, no full mania
- Cyclothymia: subthreshold hypomania + depression ≥ 2 years

MANIC EPISODE CRITERIA (DIG FAST)

- Distractibility, Impulsivity, Grandiosity
- Flight of ideas, Activity increase, Sleep decreased, Talkativeness

MOOD STABILIZERS

- Lithium: first-line, narrow therapeutic index (0.6-1.2 mEq/L)
- Lithium toxicity: tremor, polyuria, nephrogenic DI, hypothyroidism
- Valproate: teratogenic (neural tube defects), monitor LFTs
- Lamotrigine: maintenance, risk Stevens-Johnson if titrated too fast
- Atypical antipsychotics: quetiapine, aripiprazole for acute mania